



Infection Prevention and Control

Prevention and management of sharps injuries and body substance exposure policy

Introduction

Healthcare workers (HCWs) are at risk of occupational exposure to blood borne viruses, including hepatitis B virus, hepatitis C virus and immunodeficiency virus through exposure to blood, and other body fluids/substances from an infected patient.^{1,2,3} The majority of occupational exposures can be prevented with appropriate application of standard precautions and an effective sharps safety program.¹

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Hospital-Wide	All Staff

Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☒
Other:	☐		

Definitions

Body substance exposure	An exposure of blood and/or body substances to; - mucous membranes inclusive of the eyes, nose and/or mouth, or - non-intact skin, including open wound/cut, dermatitis, eczema and acne.
Occupational exposure	Incident which occurs during a work activity and involves contact with blood and/or other body substances. Exposure includes percutaneous exposure and non-percutaneous exposure. It does not include contact of blood or body substances with intact skin which would otherwise be considered a near miss event. ³
Sharps Injury (percutaneous exposure)	An injury that results in a sharp instrument/object e.g. a needle, scalpel or other sharp item, that results in cutting or puncturing of the skin. ²
Safety Engineered Device	These include: devices designed with built-in safety features to reduce the risk of injury involving a sharp. Examples include devices such as needles with guards, sliding sheaths, shields,

	blunted tips or retracting needles, blunt suture needles and surgical blades with protective covers ² • needleless devices (e.g. connectors, vascular access devices, access ports) which provide an easy access point for intravascular infusion connections or withdrawal of body substances after initial venous or arterial access is established.
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Policy principles

This policy outlines the requirements to reduce the risk to HCWs of sharps injuries and body substance exposures. It applies to all staff that use sharp instruments or come in contact with blood or body fluids.

This policy also outlines the **management** of sharps injury and body substance exposure.

1.0 Responsibilities

All HCWs are encouraged to identify and report any hazards associated with occupational exposures and be actively involved in eliminating, or where not practicable, minimising the exposure risk.

Prior to use, all HCWs are responsible for ensuring they have received appropriate training in the safe use of any sharp device, including new safety devices. The Occupational Safety and Health (OSH) department are responsible for Sharps and Body Substance Exposure education. This includes a process for review and reporting on hazard and incident reports, implementation of risk mitigation processes and training on the use of devices and handling sharps.

2.0 Safe handling and disposal of clinical sharps

All HCWs must take precautions to prevent occupational exposure from blood and body substances splashes and injuries such as those caused by:

- Needles
- Scalpels and other sharp instruments or devices during procedures and after procedures
- Disposal of used needles and sharps
- Management during cleaning used instruments
- Handling linen soiled with blood and body substances.

The SCGOPHCG sharps safety program utilises the hierarchy of controls to address and manage the risk of body fluid exposure. All staff are required to consider the application of the following principals:

1. **Eliminate unnecessary use of sharps or substitute with safer alternatives^{1,2,4}**
 - Assess the most appropriate route of medication administration and eliminate any unnecessary injections⁴
 - Mandatory use of needle-free IV access systems and blunt drawing up needles¹
 - Consider alternative cutting methods e.g. electro surgery¹
 - Use alternative suture devices e.g. stapling and glues¹
2. **Use of rigid sharps disposal containers²**
 - Discard sharps directly into a hospital designated sharps disposal container, point first, as close to the point of use as soon as possible.^{1,2}

Any person who has used a disposable sharp instrument or equipment is responsible for its safe management and immediate disposal after use.¹

3. Use of engineered controls²:

- Mandatory use of safety engineered medical devices e.g. needle-free IV access system, safety needles, safety IV cannulas, blunt drawing up needles, retractable lancets for blood glucose monitoring and the vacutainer blood collection system. Only Product Evaluation and Standardisation Committee and OSH approved safety engineered devices are to be used at SCGOPHCG.

4. Safe work practices and controls that include:

- Planning for the safe handling and disposal of sharps devices before the beginning of a procedure. This includes:
 - Assessing patient needs and explaining the procedure to prevent potential patient movement and seeking assistance for patients that may be uncooperative
 - Addressing any environmental risks including lighting, adequate space, potential distractions and potential physical hazards either during the procedure or the process of disposal
- Never recapping a used needle ^{1,2,4}
- Use of a no touch technique when handling sharps.^{1,2} This includes:
 - use of instruments rather than fingers to grasp needles, retract tissue and load/unload needles and scalpels
- Establishing a neutral safety zone to avoid sharps being passed hand to hand
- Use of clear verbal announcements when using/passing sharps
- Not bending or breaking needles by hand¹
- Ensuring contaminated sharps are organised in a standardised way, until they are disposed¹
- Containing sharps at all times, including during transport to and from the patient, using a rigid puncture proof container e.g. in a rigid kidney dish, in order to avoid injuries due to accidental disposal in linen, rubbish bags or on meal trays
- For insulin administration use a dedicated insulin injector with safety pen needle. Where patient training requires the use of a non-safety pen needle, assess and ensure patient can both self-administer insulin and safely remove and dispose of the needle prior to administration
- Assessing the need for personal protective equipment (PPE). While PPE will not prevent needlestick injuries, evidence suggests that the use of gloves, in particular double gloving during operating procedures, can reduce the risk of exposure in the event of a sharp injury²
- Never manually compress or hold rubbish/linen bags close to the body
- Reusable sharp items requiring transport to a reprocessing area must be placed in a puncture-resistant container with a lid.

3.0 Use of sharps containers

- All used sharps, including single-use syringes and needles, scalpel blades and other sharp items such as capillary tubes, glass and dental wires, are to be placed in a hospital approved and designated sharps container that conforms to Australian Standards specifications (AS 4031 – Non reusable containers for the collection of sharp medical items used in health care areas)¹
- Containers should be placed as close as practicable to the point of generation of the sharp^{1,2}

- Containers should be placed so that there is no furniture or obstacle between the site of use and the container. Wall mounted containers should be away from general waste bins and linen skips to minimise the risk of incorrect disposal.^{1,2}
- Containers in non-public access areas should be secured and placed 1.2m to the top of the sharps container from floor level to provide good visibility to the opening¹
- In public areas hospital approved lockable containers should be placed out of reach of children and a minimum of one meter from floor to top of the container
- Containers should never be free standing on the floor
- **Containers must not be filled past the capacity indicator (fill line) as marked by the manufacturer. Seal and discard sharps containers into the clinical waste bin when filled to capacity indicator.**
- Containers must not be left in patient rooms unless they are approved lockable containers secured to the wall or trolley.
- Ensure sharps containers are large enough to accommodate the device being used¹

NOTE: It is the responsibility of all staff using the sharps containers to replace them once sharps reach the designated capacity indicator (fill line) – this includes medical, nursing and allied health staff. The staff member must close/seal the container, place in the yellow clinical waste bin for incineration and replace with a new container.

4.0 Prevention of body substance exposure

Adherence to standard infection prevention practices is the first line of protection for healthcare workers against body substance exposure. Appropriate precautions include the following:

- Always ensure that personal protective equipment (PPE) is readily available or accessible
- Always undertake a risk assessment before undertaking a procedure that considers the potential risk of exposure to blood or body fluid splashes or sprays
- Use PPE e.g. gloves, apron and/or facial protection to protect mucous membranes, airways, skin and clothing from potential body substances
- Always perform hand hygiene before donning and doffing PPE
- Avoid splashing, e.g. when discarding body fluids
- Cover any cuts or abrasions with a waterproof dressing
- Apply general work safe practices including immediate attendance to body fluid spills, bagging of soiled linen prior to placing in linen skips and appropriate use of clinical waste streams

5.0 Management of sharps injuries and body substance exposure

5.1 Initial Management

Immediate first aid for the exposed person

- Thoroughly wash wound or skin site with soap and water and apply a waterproof dressing.²
- Do not squeeze or rub the injury site. ²
- Alcohol based hand rub can be used if water is unavailable.²
- If splashed in the eyes, rinse gently and thoroughly (remove contact lenses) with running water or normal saline for at least 30 seconds. If splashed in the mouth, spit out the blood or body fluid, and rinse the mouth with water several times. If any clothing is contaminated, remove and shower if necessary.

Reporting of the exposure

SCGH

1. Injured staff member (or supervisor) to immediately dial
53 (Blood and Body Substance Exposure Hotline).

This is a voice mail system. Please listen carefully to the message and respond as requested. Speak slowly and clearly leaving your full name and contact details when prompted.

Injuries will be managed:

- During hours Monday – Friday 0730 – 1500 (excluding Public Holidays) by the Occupational Health Nurse, Department of Occupational Safety and Health.
- After hours by the After Hours Clinical/Nurse Manager.

Note: If the voice mail system is not functioning:

- Dial 91 and ask to be put through to the Blood and Body Substance Exposure Hotline or
 - Page the Occupational Health Nurse (pg 4485) or
 - Contact the After Hours Clinical/Nurse Manager
SCGH: Page 4502
2. Report event to supervisor
 3. As provided by either the Occupational Health Nurse or the After Hours Clinical/Nurse Manager, complete the NMHS OSH employee hazard/incident investigation report form. Where directed by the Occupational Health Nurse or After Hours Clinical/ Nurse Manager, ensure any required blood test is completed as soon as possible.

OPH

1. Injuries will be managed:
 - During hours Monday – Friday 0630 – 1430 (excluding Public Holidays)
 - Contact the OPH Clinical Nurse Specialist Infection Prevention and Control on Ext. 78099 or page 119
 - If no response contact the SCGH Occupational Health Nurse and Health Ext. 73060.
 - After hours by the After Hours Clinical/Nurse Manager on Phone 0408 956 949
2. Report event to supervisor
3. As provided by the IPC CNS/ Occupational Health Nurse or the After Hours Clinical/Nurse Manager, complete the NMHS OSH employee hazard/incident investigation report form. Where directed by the Occupational Health Nurse or After Hours Clinical/ Nurse Manager, ensure any required blood test is completed as soon as possible.

5.2 Risk Assessment

Each exposure is assessed by the Occupational Health Nurse/IPC CNS or After-Hours Nurse Manager and on call Immunology Registrar. This includes source and recipient serology testing. Follow up is determined by an individual risk assessment by the Department of Occupational Safety and Health.

5.3 Counselling for staff

Counselling and informed consent for testing must be obtained from the recipient. For further information contact the Occupational Health Nurse Ex 73060.

5.4 Consent for testing of the source individual

The (treating) Medical Officer will counsel and obtain informed verbal consent from the source individual for blood-borne virus testing to be undertaken. This should be documented in the patient's medical record.

If the source individual is unable to give consent, attempts should be made to obtain consent from the next of kin. The Medical Officer may order tests on patients who are unable to give permission by acting as an "agent of necessity".

Related Documents

- [WA Health Healthcare Worker Immunisation Policy](#)
- [NMHS Policy Personal Protective Equipment](#)
- [NMHS Searching Patient Belongings Policy](#)
- [SCGOPHCG Nursing Practice Guideline: Diabetes Patient Management](#)
- [SCGOPHCG IP&C Manual: Standard and Transmission Based Precautions Policy](#)
- [SCGOPHCG IP&C Manual: Healthcare Worker Health and Immunisation Policy](#)
- [SCGOPHCG IP&C Manual: Reprocessing of Reusable Medical Devices](#)
- [SCGH Waste Disposal and Environmental Management Policy](#)
- [OPH Waste Management Policy](#)

References

1. Queensland Government, Queensland Health. Sharps Safety [Internet]. Queensland (Australia): The State of Queensland, 2017 [cited 2021 July 15]. Available at <https://www.health.qld.gov.au/clinical-practice/guidelines-procedures/diseases-infection/infection-prevention/standard-precautions/sharps-safety>
2. National Health and Medical Research Council. Australian guidelines for the prevention and control of infection in healthcare [Internet]. Canberra (Australia): National Health and Medical Research Council, 2019 [cited 2021 July 15]. Available at <https://www.nhmrc.gov.au/sites/default/files/documents/infection-control-guidelines-feb2020.pdf>
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5. Queensland Health. Management of occupational exposure to blood and body fluids [Internet]. Queensland (Australia): Queensland Health, 2017 [cited 2021 August 2]. Available at https://www.health.qld.gov.au/__data/assets/pdf_file/0016/151162/qh-gdl-321-8.pdf

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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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